

Clinical Documentation Sample Set

Patient Name: Michael Anderson
Date of Birth: 14/03/1981
NHS Number: 483 920 1745
Address: 72 Willow Crescent, Bradford, West Yorkshire BD5 8JT
Phone: 07700 912345
Email: michael.anderson81@examplemail.com
Hospital ID: BRD-442918
Date of Visit: 19 May 2026

Chief Complaint:

Persistent chest discomfort and shortness of breath for 3 days.

History of Present Illness:

Patient reports intermittent substernal chest tightness associated with mild exertion. Denies syncope but reports occasional dizziness. Past medical history includes hypertension and hyperlipidaemia. Current medications include Atorvastatin 20mg daily and Ramipril 5mg daily.

Assessment:

Possible unstable angina. ECG showed nonspecific ST changes. Troponin pending.

Plan:

Admit for observation, serial troponins, cardiology review, and stress testing.

Consultant: Dr. Sarah Whitmore, GMC 6072145

Referral Letter

To: Neurology Department, St. Anne's Medical Centre
Referring GP: Dr. Jonathan Reeves
Practice Address: 14 Brook Lane Surgery, Leeds LS11 4RF
Patient: Emily Carter
Date of Birth: 22/09/1992
NHS Number: 672 118 9041
Insurance Policy ID: UKP-88-12920

Dear Colleague,

I am referring Ms. Emily Carter for evaluation of chronic migraine headaches with increasing frequency over the past six months. She reports associated photophobia, nausea, and visual aura. MRI brain imaging performed on 02 April 2026 was unremarkable.

The patient currently works as a financial analyst and reports significant work-related stress. Current medications include Sumatriptan 50mg PRN and Propranolol 40mg twice daily.

Please assess and advise regarding long-term migraine management and further investigation if indicated.

Kind regards,

Dr. Jonathan Reeves
GMC 5921441

Additional Clinical Note (Page 2)

Field	Value
Patient Name	Rebecca Collins
DOB	08/12/1975
NHS Number	581 774 2283
Home Address	19 Park View Road, Manchester M14 6PL
Mobile	07911 332211
Emergency Contact	Daniel Collins - 07888 123456
Employer	Northern Telecom Services Ltd.

Progress Note:

Patient presented for follow-up regarding poorly controlled Type 2 Diabetes Mellitus. Recent HbA1c was 9.2%. Patient admits inconsistent adherence to dietary recommendations. Complains of intermittent numbness in both feet.

Lab Results:

Fasting glucose: 14.1 mmol/L

Creatinine: 92 umol/L

eGFR: 78 mL/min

Treatment Plan:

Increase Metformin to 1g BID and initiate diabetic foot care referral. Follow-up in 8 weeks.

Clinician Signature: Dr. Helen Murray